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Question 1

Not answered

Marked out of 1

Proportion of Europeans with mental health problems in past one year that attend general practitioners is

Select one:

- ☐ 50%
- ☐ 3%
- ☐ 10%
- ☐ 33%
- ☐ 67%

Your answer is incorrect.

According to ESEMeD, the first major multicentre European psychiatric epidemiological study conducted as a part of World Mental health survey initiative of WHO, only one-third of identified cases have consulted their general practitioner in preceding 12 months while 21% cases have seen a psychiatrist. Nearly one-third of those who sought help, have never seen any mental health professional.

The correct answer is: 33%

Question 2

Not answered

Marked out of 1

Which one of the following studies estimated the differences in the incidence of psychosis in different ethnic groups in UK?

Select one:

- ☐ ESEMeD survey
- ☐ AESOP study
- ☐ PRiSM Psychosis study
- ☐ UK household survey
- ☐ UK 700 study

Your answer is incorrect.

The AESOP study concluded that all psychoses are more common in the black and minority ethnic group compared to White population in Bristol, south-east London and Nottingham (crude IRR, 3.6 [95% CI, 3.0-4.2]). Differences in age, sex, and study center accounted for approximately a quarter of this effect (adjusted IRR, 2.9 [95% CI, 2.4-3.5]). The age-sex standardized incidence rate for all psychoses was higher in Southeast London (IRR, 49.4 [95% CI, 43.6-55.3]) than Nottingham (IRR, 23.9 [95% CI, 20.6-27.2]) or Bristol (IRR, 20.4 [95% CI, 15.1-25.7]) (Retrieved from <http://www.ncbi.nlm.nih.gov/pubmed/16520429>).

The correct answer is: AESOP study

Question 3

Not answered

Marked out of 1

What proportion of people seen in primary care settings has a major depressive disorder?

Select one:

- ☐ 15-20%
- ☐ 5-10%
- ☐ 10-15%
- ☐ 1-2%
- ☐ 2-3%

Your answer is incorrect.

About 5%-10% of people seen in primary care settings has a major depressive disorder Butler R, Carney S, Cipriani A, Geddes J, Hatcher S, et al. (2005) Depressive disorders. Clin Evid 14: 1-7.

The correct answer is: 5-10%

Question **4**

Not answered

Marked out of 1

According to AESOP study, which of the following groups has highest incidence rates of Bipolar Affective Disorder?

Select one:

- ☐ African Caribbean
- ☐ Black African
- ☐ Asian
- ☐ White
- ☐ Mixed ethnicity

Your answer is incorrect.

In UK, AESOP study found that African Caribbeans have the highest risk of bipolar disorder. <http://bjp.rcpsych.org/cgi/content/full/186/2/126>

The correct answer is: African Caribbean



Question 5

Not answered

Marked out of 1

The average time lag between onset and treatment of depressive disorder is

Select one:

- ☐ 2 months
- ☐ 7 years
- ☐ 5 years
- ☐ 10 years
- ☐ 3 years

Your answer is incorrect.

According to NESARC (National epidemiological survey of alcoholism and related conditions. Hasin et al. Archives 2005; 62, 1097) mean age of onset of depression is 30 years. The mean number of episodes in patients with lifetime major depressive disorder is 5. The longest duration was 24 weeks per episode. The mean age of treatment onset 33.5 years - with a lag of 3 years between onset and treatment. Women get psychiatric treatment more often and earlier than men. 50.5% of men received no treatment at all.

The correct answer is: 3 years

Question 6

Not answered

Marked out of 1

Prevalence of hallucinatory experiences in healthy British respondents from community samples is estimated to be around

Select one:

- ☐ 0.2%
- ☐ 19%
- ☐ 4%
- ☐ 33%
- ☐ 11%

Your answer is incorrect.

ONS 2000 Psychiatric morbidity survey obtained a nationally representative sample of 8580 adults aged 16-74 years living in private households in Great Britain. The respondents were interviewed by lay interviewers in 2000 (Singleton et al., 2001) and classified according to their score on the Clinical Interview Schedule - Revise Psychosis Screening Questionnaire (PSQ) was used in conjunction. 5.5% of respondents endorsed at least one psychosis item. 4.2% endorsed hallucination item (Johns, LC BJPsych 2004 185: 298-305) This 4.2% of the sample said that there had been times when they heard or saw things that other people could not, but only 0.7% reported hearing voices saying quite a few words or sentences when there was no-one around that might account for it.

The correct answer is: 4%

Question 7

Not answered

Marked out of 1

Delay in seeking psychiatric treatment is more common in all of the following situations EXCEPT

Select one:

- ☐ Being a working adult
- ☐ Living in developing country
- ☐ Having earlier age of onset
- ☐ Being > 65 years age
- ☐ Being a male

Your answer is incorrect.

From pooled analysis of WMH surveys, "median delays among cases eventually making contact was estimated from 3.0 to 30.0 years for anxiety disorders, from 1.0 to 14.0 years for mood disorders, and from 6.0 to 18.0 years for substance use disorders. Failure and delays in treatment seeking were generally greater in developing countries, older cohorts, men, and cases with earlier ages of onset. These results show that failure and delays in initial help seeking are pervasive problems worldwide" (Retrieved from <http://www.ncbi.nlm.nih.gov/pubmed/18188443>)

The correct answer is: Being a working adult

Question 8

Not answered

Marked out of 1

Prevalence of mental health problems in community surveys is always higher than the prevalence in clinical samples. Which of the following can explain this?

Select one:

- ☐ Clinical sample surveys underestimate true rates
- ☐ Difference in male and female distribution of illnesses
- ☐ Epidemiological surveys overestimate true rates
- ☐ Existence of diagnostic filters along the pathway of care
- ☐ None of the above

Your answer is incorrect.

Pathways to care - also called the filter model - was developed by Goldberg and Huxley, to account for how mental illness interacts with the healthcare system.

Five levels of mental illness occurrence were described: The community, the primary care attendees, the diagnosed primary care attendees (in whom the mental illness has been recognised), the level of psychiatrist and that at the level of psychiatric inpatient care.

Four filters explain the decreasing numbers of cases when going from the general population to inpatient psychiatric care 1. At the level of the patient himself or herself (recognition) 2. At the level of the general practitioner (recognition, decision to treat, decision to refer), 3. At the outpatient level of the mental healthcare system and 4. At the inpatient admission level. This explains the nature of hurdles one must cross before being picked up by services; this model explains why community studies show higher rates than clinic-based studies.

The correct answer is: Existence of diagnostic filters along the pathway of care

Question 9

Not answered

Marked out of 1

Low concordance with prescribing advice is predicted by

Select one:

- ☐ A short course of treatment
- ☐ Regular visits by community nurse
- ☐ Concomitant psychological treatment
- ☐ Low frequency of daily doses
- ☐ Young age of the patient

Your answer is incorrect.

Younger the patient, poorer the concordance for adult patients.

The correct answer is: Young age of the patient



Question 10

Not answered

Marked out of 1

Which of the following is not a socio-economic class recognized by the Office of National Statistics?

Select one:

- ☐ Technical
- ☐ Routine and semi-routine
- ☐ Unemployed
- ☐ Part time employed
- ☐ Professional

Your answer is incorrect.

Part time employments are not classified separately

The correct answer is: Part time employed



Question 11

Not answered

Marked out of 1

Which of the following is the strongest predictor of long duration of untreated psychosis?

Select one:

- ☐ Low educational attainment
- ☐ Minority ethnic group
- ☐ Insidious mode of onset
- ☐ High expressed emotions
- ☐ Social isolation

Your answer is incorrect.

AESOP study in the UK investigated the variables that determine long DUP. "An insidious mode of onset (psychotic symptoms appeared incrementally over a period of more than 1 month since first noticeable behavioural change) was associated with a substantially longer DUP compared with an acute onset, independent of other factors. Unemployment had a similar if less strong, effect. Conversely, family involvement in help-seeking was independently associated with a shorter duration. There was a weak suggestion (not significant on adjustment) that nonaffective psychosis was more associated with longer DUP than affective psychosis". (Excerpt from MORGAN C et al., "Clinical and social determinants of duration of untreated psychosis in the AeSOP first-episode psychosis study," The British Journal of Psychiatry 189, no. 5 (November 1, 2006): 446-452)

The correct answer is: Insidious mode of onset

Question 12

Not answered

Marked out of 1

The most crucial illness-related factor associated with long duration of untreated psychosis is

Select one:

- ☐ Family history of schizophrenia
- ☐ Presence of florid delusions
- ☐ Acute onset
- ☐ Grandiose ideas
- ☐ Negative symptoms

Your answer is incorrect.

DUP is confounded by personality and or illness-related variables, with a combination of insidious onset, negative symptoms and premorbid dysfunction contributing to delayed help-seeking, delayed initiation of treatment and poorer outcome(Retrieved from <http://bjp.rcpsych.org/content/191/50/s58>).

The correct answer is: Negative symptoms

Question 13

Not answered

Marked out of 1

Choose the most likely statement regarding prevention strategies.

Select one:

- ☐ Intervention aimed at high risk individuals is the best approach to help those individuals, but population benefits are best achieved with universal prevention
- ☐ Secondary prevention is meant to reduce the incidence of the disease by preventing the development of new cases.
- ☐ Indicated preventive intervention is an intervention targeted to members of a population with higher than average risk factor
- ☐ The goal of primary prevention to reduce the total number of existing cases by more rapid effective intervention which shortens the duration of illness
- ☐ Selective preventive intervention is targeted to an entire population.

Your answer is incorrect.

Universal preventive intervention is targeted to an entire population. Selective preventive intervention is an intervention targeted to members of a population with higher than average risk factors. Indicated preventive intervention is an intervention targeted to members of a population with subsyndromal symptoms of a disorder (or diagnosed with another associated disorder). Primary prevention is meant to reduce the incidence of the disease by preventing the development of new cases. The goal of secondary prevention is to reduce the total number of existing cases by more rapid, effective intervention, which shortens the duration of illness.

The correct answer is: Intervention aimed at high risk individuals is the best approach to help those individuals, but population benefits are best achieved with universal prevention

Question 14

Not answered

Marked out of 1

Tertiary prevention involves

Select one:

- ☐ Reducing handicap and disabilities that result from diseases
- ☐ Reducing the incidence of the diseases
- ☐ Early identification and prompt treatment of illness
- ☐ Rapid effective intervention
- ☐ Reduction of risk factors and blocking modes of disease transmission

Your answer is incorrect.

Tertiary prevention involves the reduction in the prevalence of residual defects or disabilities that are consequences of the illness. It may not be possible to eliminate the sequel of the illness fully, but the goal of tertiary prevention is for individuals to reach their highest level of functioning. In the case of schizophrenia, tertiary prevention is rehabilitation. Relapse prevention is also a form of tertiary prevention.

The correct answer is: Reducing handicap and disabilities that result from diseases

Question 15

Not answered

Marked out of 1

The MacArthur Competence Assessment Tool is most commonly used to evaluate which of the following?

Select one:

- ☐ Fitness to plead
- ☐ Parental decision making
- ☐ Withdrawal of life support
- ☐ Prisoner's capacity to vote
- ☐ Gillick Competence

Your answer is incorrect.

Hoge et al. developed and validated the MacArthur Structured Assessment of the Competence of Criminal Defendants (MacSAC- CD) to assess a defendant's fitness to plead based on the legal theory of competence. The MacSAC-CD assesses competence-related abilities and names these as Understanding, Appreciation, and Reasoning.

The correct answer is: Fitness to plead

Question 16

Not answered

Marked out of 1

Tertiary prevention is best described as a method of reducing handicap and disabilities that results from a disease. Which of the following aspects of healthcare provision is closely related to the concept of tertiary prevention?

Select one:

- ☐ Screening
- ☐ Rehabilitation
- ☐ Health education
- ☐ High risk detection
- ☐ Early intervention

Your answer is incorrect.

Tertiary preventive measures aim to reduce the burden of established disorder by optimizing treatment and rehabilitation.

The correct answer is: Rehabilitation



Question **17**

Not answered

Marked out of 1

In legal terms, which of the following regarding capacity is correct?

Select one:

- ☐ Capacity is a legal not a medical concept
- ☐ There is nothing called presumed capacity
- ☐ Capacity is generic and not task specific
- ☐ Children are presumed to lack capacity by default
- ☐ Written consent is required to demonstrate capacity

Your answer is incorrect.

Capacity is a legal, not medical concept.

The correct answer is: Capacity is a legal not a medical concept



Question 18

Not answered

Marked out of 1

You have noticed a bruise on a patient during your ward review. This bruise has not been documented so far. After physical examination and ensuring safety the most appropriate next step is

Select one:

- ☐ Inform the consultant during next weekly review
- ☐ Nothing more to do
- ☐ Inform the staff nurses and consultant as soon as possible
- ☐ Complete an incident form
- ☐ Prescribe an emollient

Your answer is incorrect.

It is important to keep members of a multi-professional team informed about the patient they care for. Though one could suspect physical abuse, jumping to conclusions with sparse information is not desirable.

The correct answer is: Inform the staff nurses and consultant as soon as possible

Question 19

Not answered

Marked out of 1

A 46-year old man with acute relapse of paranoid schizophrenia attends local A & E requesting admission to the closed ward to protect himself from cosmic rays sent by some extraterrestrial creatures. Upon admission, he refuses any medications as he feels safe in the closed ward and will not need any chemical protection. How to proceed in this case?

Select one:

- ☐ Suggest discharging him if he refuses medications
- ☐ Detain him for treatment
- ☐ Administer medications covertly
- ☐ Revise his diagnosis to malingering
- ☐ Negotiate with the patient regarding benefits of medications

Your answer is incorrect.

This is an ethically complicated scenario. The best solution remains negotiation with the patient trying to convince him of the benefit he may get from neuroleptics. Often this may mitigate the need for any further legal proceedings.

The correct answer is: Negotiate with the patient regarding benefits of medications

Question **20**

Not answered

Marked out of 1

With respect to suicide prevention, decreasing the likelihood of a suicide attempt in the high-risk patients is termed as

Select one:

- ☐ Universal prevention
- ☐ Tertiary prevention
- ☐ Ancillary prevention
- ☐ Secondary prevention
- ☐ Primary prevention

Your answer is incorrect.

This is a secondary, selective prevention method where high-risk cases e.g. all depressed patients are focussed and interventions are targeted to reduce suicides from taking place.

The correct answer is: Secondary prevention



Question **21**

Not answered

Marked out of 1

Interventions to improve prenatal nutrition and antenatal care in order to reduce the incidence of schizophrenia are best termed as

Select one:

- ☐ Tertiary prevention
- ☐ Secondary prevention
- ☐ Selected Prevention
- ☐ Indicated prevention
- ☐ Universal prevention

Your answer is incorrect.

Universal prevention measures are population based attempts to reduce incidence of a disease. Vaccinations and interventions to enhance prenatal nutrition and antenatal care are suggested as some of the plausible options for universal prevention.

Rehabilitation is an attempt to reduce disabilities and deficits rather than altering the natural course of the core illness process. It is classified as a tertiary prevention measure.

The correct answer is: Universal prevention

Question **22**

Not answered

Marked out of 1

Which of the following terms is related to confidentiality?

Select one:

- ☐ Genovese effect
- ☐ Nuremberg code
- ☐ Declaration of Helinski
- ☐ Pritchard criteria
- ☐ Tarasoff case

Your answer is incorrect.

Duty to warn refers to the responsibility of a counselor or therapist to breach confidentiality if a client or another identifiable person is in clear or imminent danger. In situations where there is clear evidence of danger to the client or other persons, the counselor must determine the degree of seriousness of the threat and notify the person in danger and others who are in a position to protect that person from harm (Herlihy and Sheeley, 1988; Pate, 1992). The legal precedent of this concept was set in the case of Tarasoff v. Regents of the University of California (1976) (Retrieved from <http://www.ncrel.org/>).

The correct answer is: Tarasoff case

Question **23**

Not answered

Marked out of 1

ECT has good evidence for treatment of severe depression though the evidence for continuation ECT is scarce. What percentage of patients will relapse despite treatment with maintenance ECT for severe depression after initial positive response?

Select one:

- ☐ One fifth
- ☐ All patients
- ☐ One tenth
- ☐ One half
- ☐ One third

Your answer is incorrect.

ECT produces a good response in formerly non-responsive patients, but relapse occurs in 33% despite continuation treatment. But note that this rate is lower than expected for this population.

Ref: Petrides G, Dhossche D, Fink M, et al: Continuation ECT: Relapse prevention in affective disorders. *Convuls Ther* 10:189-194, 1994.

The correct answer is: One third

Question **24**

Not answered

Marked out of 1

Which one among the following condition is a relative contraindication to ECT treatment?

Select one:

- ☐ Low intracranial pressure
- ☐ Past history of delirium
- ☐ Old myocardial infarction with normal rhythm.
- ☐ Pregnancy
- ☐ Previously operated vascular aneurysm

Your answer is incorrect.

Absolute contraindications are debatable: these include raised intracranial pressure, recent cerebrovascular accident, unstable vascular aneurysm and recent MI with unstable rhythm. Relative contraindications include pregnancy, retinal detachment, cerebral tumour and history of CVA.

The correct answer is: Pregnancy

Question **25**

Not answered

Marked out of 1

Which one of the following is the most common side effect of ECT treatment?

Select one:

- ☐ Drowsiness
- ☐ Headache
- ☐ Palpitations
- ☐ Weakness
- ☐ Confusion

Your answer is incorrect.

Headache is the most common side effect of ECT treatment affecting 30% of patients, and others would include weakness, confusion, drowsiness, muscular pains, palpitations and hypotension

The correct answer is: Headache



Question **26**

Not answered

Marked out of 1

Considering the social and demographic predictors of child abuse, the least likely factor from the following list is

Select one:

- ☐ Family history of schizophrenia
- ☐ History of maltreatment in parents
- ☐ Persistent crying
- ☐ Younger parental age
- ☐ History of parental substance misuse

Your answer is incorrect.

Social and demographic variables are in general limited predictors of childhood maltreatment. Substance abuse disorders are strongly associated with the onset of both abuse and neglect. Other known factors include parental history of being abused as a child, teenage parenthood and a persistently crying temperament in the baby. Depression, not schizophrenia, was a strong risk factor for childhood physical abuse. Child Abuse and Neglect, Vol 20(3), Mar 1996, 191-203.

The correct answer is: Family history of schizophrenia

Question **27**

Not answered

Marked out of 1

Which of the following is a risk factor for childhood abuse?

Select one:

- ☐ Resolved internal conflicts in the family
- ☐ Single parent family
- ☐ Family having a network of friends
- ☐ Mother's age >28
- ☐ Age of child >7

Your answer is incorrect.

Specific life situations of some families such as marital conflict, domestic violence, single parenthood, unemployment, financial stress, and social isolation may increase the likelihood of maltreatment.

The correct answer is: Single parent family



Question **28**

Not answered

Marked out of 1

A patient with bipolar illness recently recovered from a manic relapse. He wants to drive his car for personal use. How long should he remain well and stable with full insight to satisfy Driving Vehicle Licensing Agency (DVLA) criteria for relicensing?

Select one:

- ☐ 30 days
- ☐ 1 year
- ☐ 3 weeks
- ☐ 3 years
- ☐ 3 months

Your answer is incorrect.

For Group 1 (car, motorcycle) drivers with acute psychosis of any cause, hypomania/mania, schizophrenia or other chronic psychosis, driving must cease during the acute illness. Relicensing can be considered when all of the following conditions can be satisfied; 1. The patient has remained well and stable for at least 3 months 2. Is compliant with treatment 3. Is free from adverse effects of medication which would impair driving 4. Subject to a specialist favorable report 5. Regained insight in the case of bipolar mania or hypomania.

The correct answer is: 3 months

Question 29

Not answered

Marked out of 1

The Tarasoff case is an often quoted anecdote in discussions pertaining to clinical ethics. It is often invoked in the context of

Select one:

- ☐ Competency assessment
- ☐ Consent to medical treatment
- ☐ Duty to warn
- ☐ Medical negligence
- ☐ Childhood sexual abuse

Your answer is incorrect.

Tarasoff case: Prosanjit Poddar, a student in therapy at a university-based health service, expressed murderous feelings towards another student - Patricia Tarasoff. The therapist informed her supervisor and the Police but not Ms. Tarasoff. The police took no action towards safeguarding Ms. Tarasoff, and Poddar eventually murdered her. Doctors may disclose information to a third party in the best interest of the patient and the public in exceptional circumstances. This is also known as the duty to warn.

The correct answer is: Duty to warn

Question **30**

Not answered

Marked out of 1

A patient holds out his arm to have his blood pressure measured. What mode of consent has been provided to carry out the procedure?

Select one:

- ☐ Emergency
- ☐ Null consent
- ☐ Necessity
- ☐ Implied Consent
- ☐ Written consent

Your answer is incorrect.

There are some situations in which explicit consent is not needed. This would include implied consent such as when a patient holds out his arm to have his blood pressure measured (consent by consultation).

The correct answer is: Implied Consent



Question **31**

Not answered

Marked out of 1

Testamentary capacity refers to the capacity to

Select one:

- ☐ Prepare advance directives
- ☐ Appoint enduring power of attorney
- ☐ Make a valid will
- ☐ Consent to medical treatment
- ☐ Refuse treatment in dementia

Your answer is incorrect.

Capacity is task specific. One can have the capacity to choose his breakfast, but may lack the capacity to manage his estates. Testamentary capacity is the capacity to produce a valid will.

The correct answer is: Make a valid will

Question **32**

Not answered

Marked out of 1

Who is associated with transorbital leucotomy?

Select one:

- ☐ Deniker
- ☐ Klerrmann
- ☐ Rosenmann
- ☐ Jung
- ☐ Freeman

Your answer is incorrect.

Freeman performed the first transorbital lobotomy on a live patient in 1946; together with Watts, he popularised the procedure in the US.

The correct answer is: Freeman



Question **33**

Not answered

Marked out of 1

According to the DVLA, for Group 2 drivers with severe anxiety or depression, driving may be permitted when the person is well and stable for a period of

Select one:

- ☐ One year
- ☐ Three months
- ☐ Two months
- ☐ Six months
- ☐ One month

Your answer is incorrect.

Group 1 drivers with severe anxiety or depression should cease driving pending the outcome of a medical enquiry. A period of stability depending upon the circumstances will be required before driving can be resumed. For Group 2 drivers with severe anxiety or depression, driving may be permitted when the person is well and stable for a period of six months

The correct answer is: Six months

Question **34**

Not answered

Marked out of 1

For Group 1 drivers with acute psychosis of any cause, relicensing can be considered when the patient has remained well and stable for at least

Select one:

- ☐ Two years
- ☐ One year
- ☐ Three years
- ☐ Three months
- ☐ Two months

Your answer is incorrect.

Group 1 drivers: Acute psychosis of any cause (hypomania/mania, schizophrenia or other psychosis) is an indication for Group 1 drivers to cease driving; Relicensing can be considered when all of the following conditions can be satisfied; (1) The patient has remained well and stable for at least 3 months (2) Is compliant with treatment (3) Is free from adverse effects of medication which would impair driving (4) Subject to a specialist favorable report. Group 2 drivers: Driving should cease pending the outcome of a medical enquiry. The person must be well and stable for a minimum of three years with insight into their condition before driving can be resumed.

The correct answer is: Three months

Question 35

Not answered

Marked out of 1

Which one among the following is NOT an example of a statutory obligation to disclose information to a third party in the public interest?

Select one:

- ☐ In relation to communicable diseases
- ☐ Suspicion of child abuse
- ☐ Unfitness to drive
- ☐ By order of a court
- ☐ To support social benefit claims

Your answer is incorrect.

In England and Wales (and the rest of the UK), the general obligation for doctors to maintain confidentiality is a public, not a private interest. In other words, from the legal perspective, it is in the public interest for patients to be able to trust their doctors to maintain confidentiality. The issue thereof when it is lawful and when not lawful for a doctor to breach confidentiality is often a question of balancing public interests. Doctors may be obliged to disclose information to a third party in the public interest and examples are statutory obligations in relation to communicable diseases, certain controlled drugs, unfitness to drive, suspicion of child abuse or by order of a court and when there is evidence of a major crime.

The correct answer is: To support social benefit claims

Question 36

Not answered

Marked out of 1

Which of the following is true with regard to Rose's 'Prevention Paradox'?

Select one:

- ☐ An intervention aimed at high risk individuals is the best approach to help the population
- ☐ An intervention aimed at high risk individuals is the best approach to help those individuals, but population wide benefits are best achieved via universal prevention
- ☐ All universal prevention approaches are less cost effective than high risk targeted strategies
- ☐ Primary prevention cannot be achieved in a dynamically ill population
- ☐ A universal intervention is the best approach to help the individuals at a high risk of a disease

Your answer is incorrect.

In his seminal paper entitled 'Sick individuals and Sick populations', Geoffrey Rose explained the Prevention Paradox he initially put forward in 1981 using Framingham epidemiological data for coronary disease. Consider cholesterol reduction to prevent myocardial infarctions: a majority of cases of coronary disease come from a population at low or moderate cholesterol levels, and only a minority of cases come from the high-risk population. This is because the number of people at high risk is small. Similarly while advanced maternal age is a risk factor for Down's syndrome, most cases of Down Syndrome are born to younger, low-risk mothers (this is true at least in populations where most women have children at a younger age). This situation exposes the myth of equating high-risk populations with making up the majority of the disease burden. This produces a paradoxical disadvantage when implementing universal prevention programmes, as majority of individuals undergoing universal programmes will have unfavourable cost/effect ratio even though at a population level, universal preventions offer the maximum benefit

The correct answer is: An intervention aimed at high risk individuals is the best approach to help those individuals, but population wide benefits are best achieved via universal prevention

Question **37**

Not answered

Marked out of 1

Which of the following is not a primary prevention strategy for alcohol misuse?

Select one:

- ☐ Reducing availability of alcohol
- ☐ Promoting alternate leisure activities to drinking
- ☐ Promoting awareness of hazards
- ☐ Early recognition using questionnaires
- ☐ Promoting sensible drinking

Your answer is incorrect.

Secondary prevention of alcohol use includes "the use of clearly identified biological markers which would allow for the early detection of initial alcoholics. Screening programs could be implemented for selected target groups such as (a) occupational alcoholism programs, (b) programs for alcoholics' families, (c) programs for specific hospitalized patients and (d) alcohol programs related to traffic accidents" (Retrieved from <http://www.ncbi.nlm.nih.gov/pubmed/401353>).

The correct answer is: Early recognition using questionnaires

Question **38**

Not answered

Marked out of 1

Which one of the following is an essential component of recovery-oriented services?

Select one:

- ☐ Being problem based
- ☐ Being professional centred
- ☐ Being respect driven
- ☐ Being nurse directed
- ☐ Keeping the client free of responsibilities

Your answer is incorrect.

Various characters of recovery-oriented services include being self-directed, person-centred, peer supported, empowerment-based, holistic, non linear (no stepwise), strengths-based, respect driven, hope-instilling with responsibility being shared.

The correct answer is: Being respect driven

Question 39

Not answered

Marked out of 1

Which of the following is true with regard to care programme approach?

Select one:

- ☐ Involves a key initial assessment which does not need regular review
- ☐ Puts health care needs above social care needs.
- ☐ Was introduced following the killing of a social worker by a patient
- ☐ Suggests interprofessional dialogue is more important than service user's views
- ☐ Introduced in 2001

Your answer is incorrect.

The origins of the care programme approach (CPA) can be traced back to the Spokes Inquiry into the care of Sharon Campbell, who killed her social worker. (DHSS, 1988).

The correct answer is: Was introduced following the killing of a social worker by a patient

Question **40**

Not answered

Marked out of 1

The levels of care in Care Programme Approach (CPA) are divided into

Select one:

- ☐ Simple and Complex
- ☐ Levels 1, 2 and 3
- ☐ Unidisciplinary, Bidisciplinary and Multidisciplinary
- ☐ Standard and Enhanced
- ☐ Basic and Step-up

Your answer is incorrect.

Levels of CPA have been simplified in 1999 to Enhanced for those whose care needs are best served by regular multi-disciplinary review meetings and Standard where such meetings are unnecessary.

The correct answer is: Standard and Enhanced



Question **41**

Not answered

Marked out of 1

According to modernised CPA, the new name given to named professional integrating a person's health and social care needs is

Select one:

- ☐ Case manager
- ☐ Key worker
- ☐ Care coordinator
- ☐ Health provider
- ☐ Case integrator

Your answer is incorrect.

Modernizing CPA has suggested other changes that includes integrating the CPA with care management to form a single care coordination approach, appointing a lead officer to work across all agencies, applying CPA as a framework for mental health care delivery and not simply as an after-care arrangement, abolishing supervision registers and changing the name of the key worker to care coordinator.

The correct answer is: Care coordinator

Question **42**

Not answered

Marked out of 1

An assertive outreach team is taking care of a 44-year-old man with schizophrenia who despite taking his depot regularly is increasingly becoming hostile. He does not like many visitors in his flat on a weekly basis. But the outreach team decides to reduce visits to once in 2 weeks instead of withdrawing from care. This technique is called

Select one:

- ☐ Continued provision
- ☐ Unloading
- ☐ Care discontinuation
- ☐ Risk subtraction
- ☐ Backing off

Your answer is incorrect.

Backing off refers to service provision in times of need - not completely withdrawing if a patient refuses to engage.

The correct answer is: Backing off



Question **43**

Not answered

Marked out of 1

According to UK 700 study, the most important predictor of efficiency of assertive community teams is

Select one:

- ☐ Staff expertise
- ☐ Working round the clock
- ☐ Case load
- ☐ Suburban location
- ☐ Multiple professional involvement

Your answer is incorrect.

UK 700 study reported that the case load is the most important predictor of the outcome of an ACT service. ACT approach is shown to reduce admissions, reduce acute presentations at the A & E and increase compliance with secondary care. Cochrane review showed that no clear advantage could be demonstrated between ACT and intensive community management.

The correct answer is: Case load

Question **44**

Not answered

Marked out of 1

Which of the following interventions have been evaluated for early intervention of schizophrenia?

Select one:

- ☐ Use of CBT and supportive therapy
- ☐ All of the listed options
- ☐ Use of risperidone and CBT
- ☐ Use of olanzapine
- ☐ Use of specialised team input

Your answer is incorrect.

CBT may have a role in reducing progression from at risk state to full blown schizophrenia as suggested by EDIE trial (Morrison, 2004). The SoCRATES study (Study of Cognitive Reality Alignment Therapy in Early Schizophrenia) compared CBT with supportive counselling for a first- or second-episode of schizophrenia wherein at 18-month follow-up (Tarrier et al., 2004), the addition of both CBT and supportive counselling showed a significant improvement. PRIME study used olanzapine in a low dose to prevent progression with favourable results though it is not a powerful data to be generalised. EPPIC from Melbourne (McGorry et al) reported that a combination of CBT and risperidone could reduce the conversion rate at 6 months; though longer follow-up suggested that a delay not prevention effect was possible. Warner reappraised the results of EPPIC report and found out that even one additional case of schizophrenia would have nullified the much glorified results. The Lambeth Early Onset (LEO) study compared specialised care (low dose atypical anti-psychotics, CBT using manualised protocols, family counselling and vocational strategies) based on a model of assertive outreach with standard care. The remission rates were lower with specialised care.

The correct answer is: All of the listed options



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